SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Cobimetinib + vemurafenib — advanced melanoma

Advanced melanoma · NCCP 00373 · version 3 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
To control advanced BRAF V600-mutated melanoma by blocking BRAF and MEK in the same growth pathway; the two medicines have different daily schedules.

Schedule
Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Cobimetinib
MEK TARGETED TREATMENT

- ☐ Eye and retinal problems
- ☐ Reduced heart pumping function
- ☐ Muscle damage and raised muscle enzymes
- ☐ Bleeding
- ☐ Diarrhoea, rash and liver changes

Vemurafenib
BRAF TARGETED TREATMENT

- ☐ Photosensitivity, rash and new skin cancers
- ☐ Joint pain and tiredness
- ☐ Nausea, diarrhoea and dehydration
- ☐ Heart rhythm changes
- ☐ Liver, kidney and eye problems

Discuss the exact diagnosis and treatment intent, expected benefit and uncertainty, alternatives, regimen-specific risks, tests, fertility, practical burdens and your preferences. This guide supports the consent conversation and does not replace it. Only cobimetinib has the planned days 22–28 break. Vemurafenib continues. ECG measures rhythm; an echocardiogram or other LVEF test measures pumping function, and one does not replace the other. Radiation treatment needs a separate coordinated plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00373 v3.

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